Consent Policy

Practice	Kirkcaldy Dental & Implant Clinic, 23–25 Tolbooth Street, Kirkcaldy KY1 1RW
PSM section	Communication › Obtaining consent
Policy lead	Practice Owner / Principal Dentist [Nas]
Version	1.0 (draft for adoption)
Adopted	[date]
Next review	[date – 12 months after adoption]

1. Purpose

Valid consent must be obtained before any examination, investigation, treatment or personal care. This policy sets out how clinicians at Kirkcaldy Dental & Implant Clinic obtain, record and review consent, in line with Scottish law and GDC standards. It covers adults, children, adults with incapacity, refusal of consent, emergencies, and written consent for treatment including implants and private care.

2. Scope

- All dentists, dental hygienists, dental therapists, dental nurses and other registrants, including locums and trainees.
- Reception and treatment coordinator staff who give patients information about treatment and costs.
- All NHS and private patients.

The law on consent varies between UK countries. This policy reflects the law in Scotland. Staff trained elsewhere must make sure they follow Scottish law.

3. Legislation and guidance

As cited in the PSM:

- General Dental Council, *Standards for the Dental Team* (2013), Principle 3: obtain valid consent.
- Scottish Executive Health Department, *A Good Practice Guide on Consent for Health Professionals in NHS Scotland* (2006).
- Age of Legal Capacity (Scotland) Act 1991.
- Children (Scotland) Act 1995.
- Adults with Incapacity (Scotland) Act 2000.
- Mental Health (Care and Treatment) (Scotland) Act 2003.
- Smoking, Health and Social Care (Scotland) Act 2005 (dentists may sign a Section 47 Certificate of Incapacity after suitable training).
- Mental Welfare Commission for Scotland, *Right to treat?* (2022) – people who lack capacity and refuse or resist treatment.
- NHS Inform leaflets: *Consent – it's your decision*, *Information for young people using NHS services*, *Information for carers using NHS services*.

PSM pages:

- https://www.psm.sdcep.org.uk/topics/communication/obtaining-consent/
- https://www.psm.sdcep.org.uk/topics/communication/obtaining-consent/principles-of-obtaining-consent/
- https://www.psm.sdcep.org.uk/topics/communication/obtaining-consent/obtaining-consent-for-a-dental-examination-and-treatment/
- https://www.psm.sdcep.org.uk/topics/communication/obtaining-consent/children/
- https://www.psm.sdcep.org.uk/topics/communication/obtaining-consent/adults-with-incapacity/
- https://www.psm.sdcep.org.uk/topics/communication/obtaining-consent/refusal-to-provide-consent/
- https://www.psm.sdcep.org.uk/topics/communication/obtaining-consent/medical-emergencies-and-consent/
- https://www.psm.sdcep.org.uk/topics/communication/communication-with-patients/involving-the-patient-in-agreeing-their-care/

4. Responsibilities

Role	Responsibility
Practice Owner / Principal Dentist [Nas]	Makes sure this policy, consent forms and patient information are in place and up to date.
Treating clinician	Obtains valid consent for the care they provide, records it, and checks consent remains valid throughout. Consent is the responsibility of the clinician carrying out the treatment.
Dental nurses	Support the consent discussion, help the patient feel able to ask questions, and tell the clinician if the patient seems unsure or withdraws consent.
Reception / treatment coordinator [Name]	Gives accurate cost information and written estimates. Does not obtain clinical consent.
Safeguarding (Child & Adult Protection) Lead [Name]	Advises where refusal of care for a child or adult may put them at risk of harm.
Practice Manager [Name]	Keeps consent form templates current in Dentally and organises training.

5. Policy / procedure

5.1 Who can give consent

The PSM summarises that the following can consent to treatment, investigation or personal care:

- a patient aged 16 or over (presumed to have capacity);
- a patient under 16 accessing care in Scotland, if the clinician believes the child understands the nature and possible consequences of the procedure;
- a parent or carer with parental rights over a child;
- proxy consenters (in Scotland a proxy does **not** have the right to refuse treatment);
- the Court, in the case of a child.

5.2 The four principles

1. **Capacity.** The patient must be able to understand and remember what is proposed, weigh up the information (benefits, hazards and options) and use it to reach a decision. People aged 16 and over are presumed to have capacity. If the patient cannot do this, see 5.5 and contact the defence organisation for advice.
2. **Information.** Give information specific to the patient, in a way they can understand, covering:
   - the benefits of the treatment;
   - significant risks;
   - the implications of relevant options, including the option of no treatment. Give time to consider and answer questions.
3. **Freedom of choice.** The decision must be voluntary. The patient must know they can change their mind at any time.
4. **Ongoing process.** Consent is a process, not a single event. If in doubt whether the patient wishes to continue, check before proceeding and record it.

5.3 Examination, radiographs and tests

1. Before the examination explain what you plan to do. Consent may be verbal, or implied when, after the explanation, the patient sits in the chair and allows the examination. Written consent is not required for an examination.
2. Consent to an examination does not automatically include radiographs, additional diagnostic tests or referral. Explain these and obtain consent before carrying them out.
3. Record in Dentally that the examination was explained and agreed.

5.4 Treatment

1. After the examination, discuss the treatment options with the patient. Cover advantages, disadvantages and likely timescales of prevention and treatment options. Make NHS and private options clear where relevant.
2. Give a verbal or written estimate of cost.
3. Printed information about the treatment is often helpful. Give it where available.
4. Allow sufficient time to ask questions and consider options. Record the discussion.
5. Obtain **written consent** for the agreed treatment before it is carried out, using the practice consent form (based on the PSM Consent Form, Dec 2024). For this to be valid the patient must have had enough information, including costs.
6. Record and keep the consent process in the patient record.
7. Treatment under local anaesthetic: written consent is not a legal requirement, but record the consent process in the notes.
8. **Sedation or general anaesthesia: written, signed consent is required.** Keep it in the patient record.
9. Agree a long-term personal care plan with the patient, record it and give them a copy. Discuss and record any later changes.

5.5 Written consent for implants and private treatment

Implant and other private treatment is usually elective, complex and costly. The PSM requires written consent for agreed treatment, with sufficient information including costs. For implants and other private treatment the clinician must therefore:

1. Hold an assessment and discussion appointment before the treatment appointment [practice decision: minimum interval between consent discussion and surgery, e.g. a "cooling-off" period].
2. Give a written treatment plan and written estimate of fees, including what is and is not included (for example reviews, maintenance, and what happens if an implant fails) [practice decision: implant warranty/aftercare terms].
3. Explain, and record, the benefits, significant risks and the alternatives, including NHS options where available and no treatment.
4. Give printed information about the procedure and aftercare.
5. Obtain a signed consent form specific to the procedure before treatment starts [practice decision: implant-specific consent form, adapted from the PSM Consent Form].
6. Re-confirm consent on the day of surgery and record it. If the plan changes, discuss and record new consent.
7. If sedation is used, obtain separate written, signed consent for sedation.

5.6 Children

1. Under the Age of Legal Capacity (Scotland) Act 1991, young people aged 16 have legal capacity unless they lack mental capacity.
2. A child under 16 can consent for themselves if the clinician believes they understand the nature and possible consequences of the treatment. This depends on age, maturity and the complexity, likely outcome and risks of the treatment. Record the assessment.
3. Consider the child's own views, not just those of the parent or carer.
4. If the child cannot understand, ask the parent or carer for consent.
5. Where a child is old enough to understand an explanation but no parent or carer is present, do not carry out irreversible treatment (such as an extraction) until parental consent is available.
6. For patients **under 12**, it would be unwise to proceed on the consent of the child alone.
7. If no parent or carer is available, under the Children (Scotland) Act 1995 a person who has care or control of the child but no parental responsibilities (e.g. step-parent, relative, childminder) may consent if they believe the parent would not refuse. This does **not** include teachers or others caring for the child at school.
8. If an older child (e.g. 15) who understands refuses parental involvement, the clinician may proceed, documenting all discussions carefully. The child's capacity to understand the consequences is the key factor.
9. Reception records parent/carer details at registration and asks who will accompany the child.

5.7 Adults with incapacity

1. Most people with a mental disorder or learning disability can make some healthcare decisions. Capacity depends on the decision, the person's state at the time and the nature of the condition.
2. Under the Adults with Incapacity (Scotland) Act 2000, incapacity means being incapable of acting, making, communicating, understanding or retaining the memory of decisions, because of mental disorder or inability to communicate due to physical disability (unless this can be overcome by human or mechanical aid).
3. Support the patient to exercise their right to consent wherever possible and use any residual capacity. Give explanations in a suitable way (e.g. plain language, visual aids, interpreter).

4. If there is doubt about understanding, capacity must be assessed. Contact a responsible relative or carer to discuss.
5. Check whether a **Welfare Attorney** (or guardian) has been appointed. Liaise with them before treatment. A Section 47 Certificate is still needed.
6. A suitably trained and qualified dentist can assess capacity and issue a **Certificate of Incapacity (Section 47 Certificate)** for dental treatment. NES provides training. [Practice decision: list dentists in the practice who have completed NES AWI training.]
7. If no dentist in the practice is trained to issue a certificate, ask a doctor (e.g. the patient's GP) to assess the patient and issue a certificate for dental treatment.
8. Be cautious about significant treatment (for example extracting the last few teeth or front teeth) without input from relatives.
9. Special rules apply where an adult with incapacity refuses or resists treatment. Follow the Mental Welfare Commission *Right to treat?* guidance and seek advice.
10. Make the NHS Inform leaflet *Information for carers using NHS services* available.
11. Keep a copy of the Section 47 Certificate and any power of attorney details in Dentally.

5.8 Refusal of consent

1. A patient with capacity aged 16 or over has the right to refuse care.
2. Do not continue if a patient with capacity refuses or withdraws valid consent. Treating without consent can lead to criminal charges, civil action and allegations of professional misconduct.
3. Record the refusal, the information given and the risks explained.
4. If a child, or the carer of a child, refuses consent or will not co-operate, try again at a later appointment.
5. If consent is still not given and the child needs urgent attention or is at risk of harm, inform the named contact in the local child protection procedures (NHS Fife) immediately, via the Safeguarding Lead. See the safeguarding policy.
6. If in doubt, contact the dental defence organisation [Name].

5.9 Medical emergencies

1. Obtain consent from the patient if possible.
2. If the patient cannot give or refuse consent, it is usually acceptable to give treatment that is immediately necessary to save life or avoid significant deterioration in health.
3. Give no more treatment than the immediate situation requires.
4. Record what was done and why.

5.10 When in doubt

If in doubt about any aspect of consent, seek advice from the dental defence organisation before starting examination or treatment.

6. Records and monitoring

- Every course of treatment has a consent record in Dentally: the options discussed, risks, costs, the patient's decision and who gave consent.
- Signed consent forms (treatment, implants, sedation) are scanned or completed digitally in Dentally and kept with the patient record.

- For children and adults with incapacity, record the capacity assessment, who consented and on what basis, and any Section 47 Certificate.
- Record any refusal or withdrawal of consent.
- [Practice decision: audit a sample of [10] records per clinician every [12] months for consent documentation, as a quality improvement activity. Discuss results at a staff meeting.]

7. Training

- All clinicians receive training on this policy at induction.
- Clinicians keep up to date on consent law through CPD.
- Dentists who will issue Section 47 Certificates complete NES Dental Adults with Incapacity training.
- Staff are aware of the Scotland-specific rules on children and proxy consent.

8. Related policies and templates

PSM templates to download and adapt:

- Consent Form (Dec 2024) (Templates › Communication) – adapt versions for implants and sedation
- Patient Review and Personal Care Plan (SDCEP Oral Health Assessment and Review)
- NHS Inform: *Consent – it's your decision*; *Information for young people using NHS services*; *Information for carers using NHS services*

Related policies in this set:

- Policy 06 – Referrals and Communication with Other Professionals Protocol
- Policy 01 – Feedback, Comments, Concerns and Complaints Procedure
- Policy 02 – Duty of Candour Procedure
- 12 Child and adult protection and 34 Medical emergencies

9. Inspection checklist

- ☐ A consent policy based on GDC and Scottish Government guidance is in place.
- ☐ Written consent forms are used for agreed treatment and kept in the patient record.
- ☐ Written, signed consent is obtained for sedation or general anaesthesia.
- ☐ Records show the options, risks, benefits and costs discussed.
- ☐ Staff understand consent rules for children under the Age of Legal Capacity (Scotland) Act 1991 and the Children (Scotland) Act 1995.
- ☐ Staff understand the Adults with Incapacity (Scotland) Act 2000 and Section 47 Certificates; trained dentists are identified.
- ☐ Patient information on consent (NHS Inform leaflets) is available.
- ☐ Personal care plans are agreed, recorded and given to patients.

Staff sign-off

Name	Role	Signature	Date

Name	Role	Signature	Date